

NEXUS CLINICAL LABORATORIES

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LABORATORY REPORT

Patient: Isabella Santos	DOB: December 18, 1971 (Age 54)
MRN: MRN-9910914	Gender: Female
Ordering Physician: Dr. Michael Chen, MD — Gastroenterology	Accession #: NX-67585874
Specimen Type: Venous Blood / Urine	Collected: May 04, 2025 at 12:00 AM
Received: May 02, 2025 at 02:00 AM	Reported: June 21, 2025 at 05:00 AM

COMPLETE BLOOD COUNT (CBC) WITH DIFFERENTIAL

Test	Result	Units	Reference Range	Flag
WBC — White Blood Cell Count	6.8	x10 ³ /μL	4.5 – 11.0	
RBC — Red Blood Cell Count	5.12	x10 ⁶ /μL	3.80 – 5.10	
Hemoglobin	13.4	g/dL	12.0 – 16.0	
Hematocrit	35.8	%	36.0 – 46.0	L
MCV — Mean Corpuscular Volume	81.2	fL	80.0 – 100.0	
MCH	30.6	pg	27.0 – 33.0	
Platelets	260	x10 ³ /μL	150 – 400	
Neutrophils	55.1	%	50.0 – 70.0	
Lymphocytes	21.6	%	20.0 – 40.0	

COMPREHENSIVE METABOLIC PANEL (CMP)

Test	Result	Units	Reference Range	Flag
Glucose (Fasting)	109	mg/dL	70 – 99	H
BUN — Blood Urea Nitrogen	10	mg/dL	7 – 25	
Creatinine	0.7	mg/dL	0.50 – 1.10	
eGFR	79	mL/min	> 60	
Sodium	141	mEq/L	136 – 145	
Potassium	4.6	mEq/L	3.5 – 5.0	
Calcium	9.8	mg/dL	8.5 – 10.2	
AST	16	U/L	10 – 40	
ALT	34	U/L	7 – 56	
Total Bilirubin	0.6	mg/dL	0.1 – 1.2	
Albumin	4.2	g/dL	3.5 – 5.0	

THYROID FUNCTION PANEL

Test	Result	Units	Reference Range	Flag
TSH — Thyroid Stimulating Hormone	3.29	mIU/L	0.45 – 4.50	
Free T4 (FT4)	0.95	ng/dL	0.82 – 1.77	
Free T3 (FT3)	3.0	pg/mL	2.0 – 4.4	

CLINICAL INTERPRETATION / COMMENTS

Findings are consistent with microcytic hypochromic anemia, likely secondary to iron deficiency; correlation with serum iron studies (ferritin, TIBC) is recommended. Mildly elevated fasting glucose warrants an HbA1c to evaluate for pre-diabetes.

Reviewed & Verified by: Dr. Michael Chen, MD | Pathology & Lab Medicine | June 21, 2025 at 05:00 AM CST | Report ID: NX-67585874